

Tremfya (guselkumab)
Coverage Policy

Policy Title: Clinical Review Criteria for Tremfya (guselkumab)	Effective Date: 1/1/2018
Policy Number: PS645POLcm	Revision Date: 6/17/2025
Lines of Business: Commercial	

HNE Approved Indications for use

Tremfya (guselkumab) is an interleukin-23 blocker indicated for the treatment of adult patients with:

- Moderate-to-severe plaque psoriasis who are candidates for systemic therapy or phototherapy
- Active psoriatic arthritis
- Moderately to severely active ulcerative colitis
- Moderately to severely active Crohn's disease

The following criteria must be met for coverage of Tremfya (guselkumab):

Criteria for coverage:

Plaque Psoriasis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a dermatologist **AND**
- Diagnosis of moderate to severe chronic plaque psoriasis involving $\geq 10\%$ of body surface area **AND**
- Member has had a minimum duration of a 4-week trial and failure, or intolerance to **ONE** of the following topical therapies or contraindication to **ALL** of them:
 - Corticosteroids
 - Vitamin D analogs
 - Tazarotene
 - Calcineurin inhibitors
 - Anthralin
 - Coal tar **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), upadacitinib (Rinvoq), certolizumab (Cimzia), etanercept (Enbrel), infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), baricitinib (Olumiant)]

Psoriatic Arthritis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a dermatologist or rheumatologist **AND**
- Diagnosis of active psoriatic arthritis **AND**

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- Member must be intolerant or failed a trial of at least **ONE** (1) of the following: a DMARD (i.e., methotrexate, sulfasalazine, hydroxychloroquine, aurothioglucose, auranofin, gold sodium thiomalate, azathioprine, d-penicillamine, cyclosporine, leflunomide **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), upadacitinib (Rinvoq), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), baricitinib (Olumiant)]

Ulcerative colitis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of moderate to severe ulcerative colitis **AND**
- Member has tried and failed or was intolerant to at least **ONE** of the following conventional therapies or has a contraindication to **ALL** of the following:
 - 6-mercaptopurine **OR**
 - An aminosalicylate (e.g. mesalamine, olsalazine, sulfasalazine) **OR**
 - Azathioprine **OR**
 - an oral corticosteroid (e.g. prednisone) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), ixekizumab (Taltz), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), golimumab (Simponi/Simponi Aria), baricitinib (Olumiant), tofacitinib (Xeljanz or Xeljanz XR)]

Crohn's disease

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of moderate to severe Crohn's disease **AND**
- Member has tried and failed or was intolerant to at least **ONE** of the following conventional therapies or has a contraindication to **ALL** of the following:
 - 6-mercaptopurine **OR**
 - Azathioprine **OR**
 - an oral corticosteroid (e.g. prednisone) **OR**
 - methotrexate **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), ixekizumab (Taltz), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab

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(Kevzara), natalizumab (Tysabri), golimumab (Simponi/Simponi Aria), baricitinib (Olumiant), tofacitinib (Xeljanz or Xeljanz XR)]

Criteria for continuation of therapy:

Plaque Psoriasis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a dermatologist **AND**
- Diagnosis of plaque psoriasis **AND**
- Documentation of positive clinical response to Tremfya therapy (e.g., labs, sign/symptom reduction, no disease progression, no significant toxicity) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), upadacitinib (Rinvoq), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), baricitinib (Olumiant)]

Psoriatic Arthritis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a dermatologist or rheumatologist **AND**
- Diagnosis of psoriatic arthritis **AND**
- Documentation of positive clinical response to Tremfya therapy (e.g., labs, sign/symptom reduction, no disease progression, no significant toxicity) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), upadacitinib (Rinvoq), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), baricitinib (Olumiant)]

Ulcerative colitis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of ulcerative colitis **AND**
- Member demonstrated a clinical response to therapy **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), baricitinib (Olumiant)]

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Crohn's disease

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of moderate to severe Crohn's disease **AND**
- Member demonstrated a clinical response to therapy **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), adalimumab (Humira), adalimumab-atto (Amjevita), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), apremilast (Otezla), ustekinumab (Stelara), tocilizumab (Actemra), secukinumab (Cosentyx), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), baricitinib (Olumiant)]

Dosing recommendations:

Plaque psoriasis

- The recommended dose is 100 mg at Week 0, Week 4, and every 8 weeks thereafter.
- Available as a prefilled syringe or one-press patient controlled injector

Psoriatic arthritis

- The recommended dose is 100 mg at Week 0, Week 4, and every 8 weeks thereafter.

Ulcerative colitis

- Induction: 200 mg administered by intravenous infusion over at least one hour at Week 0, Week 4, and Week 8
- Maintenance:
 - 100 mg administered by subcutaneous injection at Week 16, and every 8 weeks thereafter, or
 - 200 mg administered by subcutaneous injection at Week 12, and every 4 weeks thereafter.
 - Use the lowest effective recommended dosage to maintain therapeutic response.

Crohn's disease

- Induction:
 - 200 mg administered by intravenous infusion over at least one hour at Week 0, Week 4, and Week 8, or
 - 400 mg administered by subcutaneous injection (given as two consecutive injections of 200 mg each) at Week 0, Week 4, and Week 8.
- Maintenance
 - 100 mg administered by subcutaneous injection at Week 16, and every 8 weeks thereafter, or
 - 200 mg administered by subcutaneous injection at Week 12, and every 4 weeks thereafter
 - Use the lowest effective recommended dosage to maintain therapeutic response.

Quantity Limits and duration:

- Initial therapy: Approve for 6 months, quantity limit:

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- *Plaque psoriasis and psoriatic arthritis*: 100mg strength only: 2/28 for the first month then 1/56
- *Ulcerative Colitis*:
 - IV: 200 mg vial: 2 for the first month, 1 for month 2
 - Subq: 200mg: 1 inj per month, 100mg: 1 inj every 2 months
- *Crohn's disease*:
 - IV: 200 mg vial: 2 for the first month, 1 for month 2
 - Subq: 200mg: 4 inj for the 1st month, 2 inj for the 2nd month, then 1 inj per month, 100mg: 1 inj every 2 months
- Continuation of therapy: Approve for 1 year, quantity limit
 - *Plaque psoriasis and psoriatic arthritis*: 100mg strength only 1/56
 - *Ulcerative Colitis*:
 - Subq: 200mg: 1 inj per month, 100mg: 1 inj every 2 months
 - *Crohn's disease*:
 - Subq: 200mg: 1 inj per month, 100mg: 1 inj every 2 months

References:

1. Tremfya (Prescribing Information). Horsham, PA: Janssen Biotech, Inc; 2025 March.
2. Blauvelt A, Papp KA, Griffiths CE, et al. Efficacy and safety of guselkumab, an anti-interleukin-23 monoclonal antibody, compared with adalimumab for the continuous treatment of patients with moderate to severe psoriasis: results from the phase III, double-blinded, placebo- and active comparator-controlled VOYAGE 1 trial. *J Am Acad Dermatol*. 2017;76(3):405-417.
3. Reich K, Armstrong AW, Foley P, et al. Efficacy and safety of guselkumab, an anti-interleukin-23 monoclonal antibody, compared with adalimumab for the treatment of patients with moderate to severe psoriasis with randomized withdrawal and retreatment: results from the phase III, double-blind, placebo- and active comparator-controlled VOYAGE 2 trial. *J Am Acad Dermatol*. 2017;76(3):418-431.
4. Hsu S, Papp KA, Lebwohl MG, et al. Consensus guidelines for the management of plaque psoriasis. *Arch Dermatol*. 2012;148(1):95-102.
5. Furst DE, Keystone EC, So AK, et al. Updated consensus statement on biological agents for the treatment of rheumatic diseases, 2012. *Ann Rheum Dis*. 2013;72 Suppl 2:ii2-34.
6. Nakamura M, Lee K, Jeon C et al. Guselkumab for the Treatment of Psoriasis: A Review of Phase III Trials. *Dermatol Ther (Heidelb)*. 2017 Jun 21. doi: 10.1007/s13555-017-0187- 0.